



Scenario Clinic Capture

Use this template during the scenario clinic section of the **Metrics Based Process Mapping** workshop. The purpose of the scenario clinic is to test a real process or automation idea using the workshop method. The goal is not to fully solve the process in the room — the goal is to understand the opportunity well enough to decide the next best action.

📖 MAP THE WORK

🔧 MEASURE THE PAIN

⚡ AUTOMATE WITH PURPOSE

When to Use This Template

This template is designed for specific moments in the workshop process. Use it in the following situations:

Running the Scenario Clinic

Use during the scenario clinic section of the Metrics Based Process Mapping workshop.

Testing a Candidate Idea

Use when testing a candidate automation idea with a group in a structured way.

Making a Decision

Use when deciding whether a process should be automated, improved, investigated, or deferred.

Capturing Assumptions

Use when capturing assumptions, missing evidence, and follow-up actions from the discussion.

Creating a Useful Record

Use when turning a workshop discussion into a structured, actionable record for the team.

How to Use This Template

Follow these steps in order during the scenario clinic session. Keep the discussion at **discovery level** — do not try to design the full automation solution in the session.

O1	O2	O3
Select one scenario or candidate process	Define where the process starts and ends	Identify the pain points
O4	O5	O6
Capture known evidence and assumptions	Discuss whether the likely response is automation, improvement or both	Categorise the opportunity
O7		
Agree the next best action		

1. Session Details

Complete this section at the start of the session to establish context and ownership.

Session name	[SESSION NAME]
Date	[DATE]
Facilitator	[FACILITATOR]
Team / group	[TEAM]
Scenario owner	[SCENARIO OWNER]
Participants	[PARTICIPANTS]
Related workshop / deck	[REFERENCE]

2. Scenario Summary

Capture a brief description of the scenario being discussed. This sets the context for the rest of the clinic.

Scenario / process name	[PROCESS NAME]
Short description	[ONE OR TWO SENTENCES DESCRIBING THE PROCESS]
Why this was selected	[WHY IS THIS WORTH DISCUSSING?]
Current automation idea, if any	[DESCRIBE CURRENT IDEA OR LEAVE BLANK]

 Keep the scenario description brief. One or two sentences is enough at this stage. Detail comes later.

3. Scope the Process

Define the scope **before** discussing solutions. Use the table below to establish clear boundaries for the process being examined.

What starts the process?	[TRIGGER]
Where does the process begin?	[START POINT]
Where does the process end?	[END POINT]
What outcome is needed?	[DESIRED OUTCOME]
What is out of scope?	[OUT OF SCOPE]

Facilitator prompt: Before we decide whether this should be automated, let's define the process we are actually talking about.

4. Who and What Is Involved?

People, Roles and Teams

Role / team	Involvement	Notes
[ROLE / TEAM]	[WHAT THEY DO]	[NOTES]
[ROLE / TEAM]	[WHAT THEY DO]	[NOTES]
[ROLE / TEAM]	[WHAT THEY DO]	[NOTES]

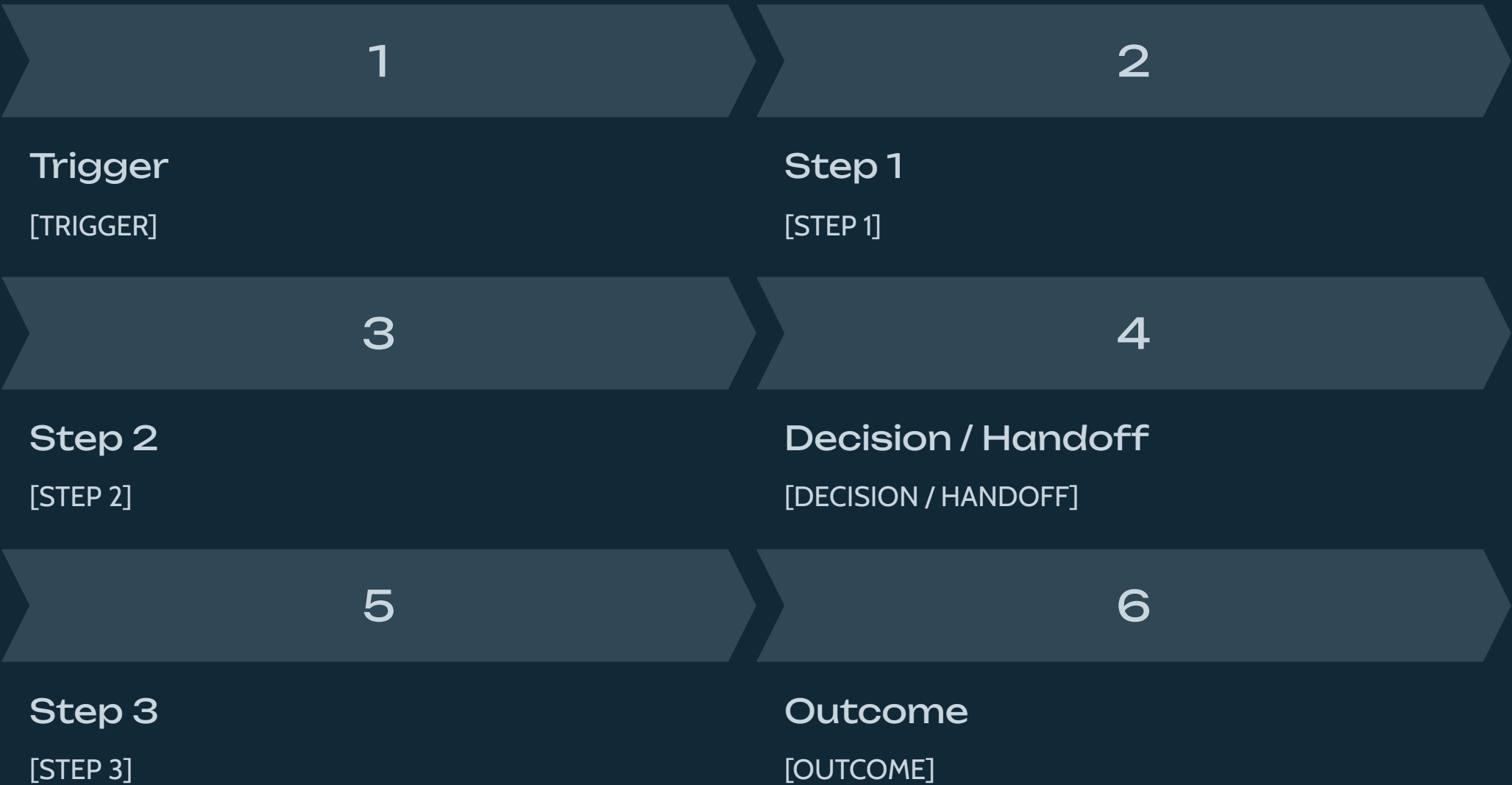
Systems, Tools, Forms and Documents

System / tool / document	How it is used	Notes
[SYSTEM / TOOL]	[HOW IT IS USED]	[NOTES]
[SYSTEM / TOOL]	[HOW IT IS USED]	[NOTES]
[SYSTEM / TOOL]	[HOW IT IS USED]	[NOTES]

Facilitator prompts: Who touches this process? Who approves, reviews or owns decisions? What tools, forms, spreadsheets or systems are used? Where does information move between people or systems?

5. Simple Process Flow

Capture the main flow at a high level. Start with the normal path. Add exceptions later if needed.



Notes

[Capture any useful notes about the flow.]

Facilitator prompt: Let's keep this at the normal-path level first. We can add exceptions once the main flow is clear.

6. Pain Points

Capture where the process creates effort, delay, errors, risk or poor experience.

Pain point	Where it occurs	Who is affected?	Impact
[PAIN POINT]	[STEP / HANDOFF / DECISION]	[WHO IS AFFECTED?]	[IMPACT]
[PAIN POINT]	[STEP / HANDOFF / DECISION]	[WHO IS AFFECTED?]	[IMPACT]
[PAIN POINT]	[STEP / HANDOFF / DECISION]	[WHO IS AFFECTED?]	[IMPACT]

Pain Type — Select all that apply



Effort

Too much manual work, repeated activity or low-value effort



Flow

Delay, waiting, bottlenecks, handoffs or poor visibility



Quality

Errors, rework, missing information or inconsistent outputs



Risk

Compliance, control, security, audit or operational risk



User Impact

Poor experience, frustration, chasing or reduced productivity



Cost

Avoidable cost, duplication or inefficient use of resources

Facilitator prompts: Where does the process slow down? Where is manual effort involved? What tends to go wrong? Where does rework happen? Who experiences the pain? What happens if this process fails?

7. Evidence and Metrics

Capture known evidence, estimates and missing metrics. Estimates are fine, but mark them clearly.

Metric / evidence	Known value or estimate	Source	Confidence	Notes
Volume	[VALUE]	[SOURCE]	High / Medium / Low	[NOTES]
Cycle time	[VALUE]	[SOURCE]	High / Medium / Low	[NOTES]
Touch time	[VALUE]	[SOURCE]	High / Medium / Low	[NOTES]
Wait time	[VALUE]	[SOURCE]	High / Medium / Low	[NOTES]
Handoffs	[VALUE]	[SOURCE]	High / Medium / Low	[NOTES]
Rework / errors	[VALUE]	[SOURCE]	High / Medium / Low	[NOTES]
Exceptions	[VALUE]	[SOURCE]	High / Medium / Low	[NOTES]
Risk impact	[VALUE]	[SOURCE]	High / Medium / Low	[NOTES]
User impact	[VALUE]	[SOURCE]	High / Medium / Low	[NOTES]

Facilitator prompts: What do we know? What are we estimating? What are we assuming? What data would help us decide whether this is worth taking forward? Who could help provide the missing evidence?

8. Assumptions and Unknowns

Capture assumptions separately from known evidence. Both are useful, but they should not be treated the same way.

Assumption / unknown	Why it matters	How to validate	Owner
[ASSUMPTION / UNKNOWN]	[WHY IT MATTERS]	[VALIDATION ACTION]	[OWNER]
[ASSUMPTION / UNKNOWN]	[WHY IT MATTERS]	[VALIDATION ACTION]	[OWNER]
[ASSUMPTION / UNKNOWN]	[WHY IT MATTERS]	[VALIDATION ACTION]	[OWNER]

 Do not treat assumptions as facts. Capture them here so they can be validated before decisions are made.

Facilitator prompt: Let's separate what we know from what we are assuming. Both are useful, but they should not be treated the same way.

9. Likely Response

Based on the discussion so far, what kind of response appears most appropriate? Select one or more options below.

Response Options — Select all that apply

- Automation
- Process improvement
- Automation and process improvement
- Standardisation
- Policy / rule clarification
- Data quality improvement
- Workflow visibility / reporting
- Further technical investigation
- Defer for now
- Other — [DESCRIBE]

Why?

[Explain why this response appears appropriate based on the discussion.]

Facilitator Prompts

- Is this really an automation problem?
- What needs to be improved before automation would make sense?
- Is the pain caused by manual effort, waiting, rework, unclear rules or something else?
- Would automation solve the pain, or just move it somewhere else?

10. Value, Feasibility and Readiness Check

Use a quick assessment to test the opportunity across three lenses.

Lens	Key question	Assessment	Notes
Value	Is this worth solving?	High / Medium / Low / Unknown	[NOTES]
Feasibility	Can we automate it?	High / Medium / Low / Unknown	[NOTES]
Readiness	Is the process ready?	High / Medium / Low / Unknown	[NOTES]

Value Prompts

- How significant is the pain?
- How often does this happen?
- Who benefits if this improves?
- What risk, delay, rework or effort could be reduced?

Feasibility Prompts

- Are the rules clear?
- Are inputs and outputs stable?
- Is the data accessible?
- Are systems accessible through workflow, integration or automation tooling?
- Are exceptions manageable?

Readiness Prompts

- Is there a process owner?
- Is the process stable?
- Are stakeholders aligned?
- Are approval or decision rules clear?
- Are there dependencies to resolve first?

11. Opportunity Category

Select the category that best describes the opportunity based on the discussion.

Automate Now

High value, feasible and ready

Typical next action: Move into automation discovery or design

Improve First

Worth solving, but process needs tidying

Typical next action: Clarify, simplify, standardise or assign ownership

Investigate Further

Potential value, but more evidence is needed

Typical next action: Gather data or assess technical feasibility

Do Not Automate Yet

Low value, unstable or unclear

Typical next action: Defer or address underlying process issues

Selected Category

☐ Automate now ☐ Improve first ☐ Investigate further ☐ Do not automate yet

Rationale

[Explain why this category was selected.]

Facilitator prompt: Where does this land today based on what we know? This can change later as more evidence becomes available.

12. Next Best Action

Capture the next best action clearly. **The output is not a final solution. The output is the next best action.**

Action	Owner	Due date	Notes
[NEXT ACTION]	[OWNER]	[DATE]	[NOTES]
[NEXT ACTION]	[OWNER]	[DATE]	[NOTES]
[NEXT ACTION]	[OWNER]	[DATE]	[NOTES]

Examples of Next Best Actions

- Validate the process with people who do the work
- Gather actual volume data
- Estimate current touch time
- Measure wait time or approval delay
- Clarify process ownership
- Define the standard path and exceptions
- Assess system integration options
- Improve the intake form
- Standardise inputs or decision rules
- Add the opportunity to the backlog

Facilitator prompt: What is the smallest useful next step that will help us make a better decision?

13. Scenario Summary

Use this section to summarise the scenario in a few lines once the clinic discussion is complete.

Scenario / process	[PROCESS NAME]
Main pain	[SUMMARY]
Known evidence	[SUMMARY]
Missing evidence	[SUMMARY]
Likely response	Automation / Improvement / Both / Investigate further / Defer
Value	High / Medium / Low / Unknown
Feasibility	High / Medium / Low / Unknown
Readiness	High / Medium / Low / Unknown
Category	Automate now / Improve first / Investigate further / Do not automate yet
Next best action	[SUMMARY]
Owner	[OWNER]

✔ This summary is the key output of the scenario clinic. Share it with participants after the session as the record of what was discussed and agreed.

14. Parking Lot

Use this section to capture items that are useful but outside the current discussion. These should be revisited after the session.

Item	Why it matters	Follow-up owner
[PARKING LOT ITEM]	[WHY IT MATTERS]	[OWNER]
[PARKING LOT ITEM]	[WHY IT MATTERS]	[OWNER]
[PARKING LOT ITEM]	[WHY IT MATTERS]	[OWNER]

- ❏ The parking lot keeps the session focused. Capture items here rather than letting them derail the discussion — they can be followed up separately.

15. Capture Checklist

Before closing the scenario, check that the following have been captured. Use this as a final quality check before ending the session.

- | | |
|--|---|
| ☐ Process name | ☐ Missing evidence |
| ☐ Start point | ☐ Assumptions |
| ☐ End point | ☐ Likely response |
| ☐ Desired outcome | ☐ Value / feasibility / readiness view |
| ☐ Key roles / teams | ☐ Opportunity category |
| ☐ Systems or tools used | ☐ Next best action |
| ☐ Main pain points | ☐ Owner |
| ☐ Known evidence or estimates | |

✓ If all items are checked, the scenario clinic is complete. The record is ready to share.

Final Reminder

The scenario clinic is not about solving everything in the room. It is about answering five key questions clearly and confidently.

1 What is the process?

Define the scope, trigger, start, end and desired outcome before anything else.

2 Where is the pain?

Identify the effort, flow, quality, risk, user impact or cost issues in the process.

3 What evidence do we have?

Capture known metrics, estimates and assumptions — and be clear about which is which.

4 Is the likely response automation, improvement or both?

Categorise the opportunity honestly based on value, feasibility and readiness.

5 What is the next best action?

Agree the smallest useful next step that will help the team make a better decision.

 MAP THE WORK

 MEASURE THE PAIN

 AUTOMATE WITH PURPOSE